

The Lehigh Valley Crisis: The UnitedHealthcare and LVHN Network Fracture

In Rating Area 6 and Lehigh County, the commercial affordability crisis is exacerbated by a severe network disruption. Following a prolonged, two-year contract dispute, Lehigh Valley Health Network (now a wholly owned subsidiary of the 32-hospital Jefferson Health system) terminated its contract with UnitedHealthcare. This network fracture has unfolded across two distinct operational timelines :

- **The Medicare Advantage Disruption:** On January 26, 2026, Lehigh Valley Health Network officially exited UnitedHealthcare's Medicare Advantage network, affecting over 20,000 senior citizens in the region and causing approximately 5,400 active UnitedHealthcare Medicare Advantage policyholders to immediately lose in-network access to their established Lehigh Valley Health Network primary care physicians and specialists.
- **The Commercial and Employer-Plan Disruption:** On April 26, 2026, the contract covering UnitedHealthcare employer-sponsored commercial plans, individual marketplace plans, and the Veterans Affairs Community Care Network expired, displacing approximately 50,000 active commercial enrollees in the Lehigh Valley.

An important legal and operational exception exists regarding the Veterans Affairs Community Care Network. Although early communications suggested that Veterans enrolled in the UnitedHealthcare-administered Veterans Affairs Community Care Network would face the same out-of-network transition as commercial enrollees, subsequent joint statements from UnitedHealthcare and Lehigh Valley Health Network confirmed that Veterans are legally exempt. Veterans may continue to access all Lehigh Valley Health Network facilities and clinicians under the network without interruption. Similarly, UnitedHealthcare's state-administered Medicaid and Children's Health Insurance Program plans remain fully in network.

The contract breakdown is characterized by conflicting public narratives from both corporate entities :

- **UnitedHealthcare's Actuarial Argument:** UnitedHealthcare publicly positions Lehigh Valley Health Network as "one of the most expensive health systems in eastern Pennsylvania". Insurer executives allege that Lehigh Valley Health Network deliberately stalled contract negotiations for over six months, using the highly sensitive Medicare Annual Enrollment Period as leverage to demand an unsustainable, one-year price hike exceeding 20% for its commercial plans. To justify its resistance, UnitedHealthcare published localized price comparisons, asserting that routine diagnostic and clinical services at Lehigh Valley Health Network are priced significantly above regional averages

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$\text{Excess CT/MRI Cost at LVHN} = +\$1,800$ $\text{Excess ER Visit Cost} = +\$1,100$
 $\text{Excess Total Hip Replacement Cost} = +\$24,000$

UnitedHealthcare argues that agreeing to these rates would force local employers to absorb massive premium increases, reducing capital available for employee wages and business expansion.

- **Lehigh Valley Health Network's Clinical Argument:** Conversely, Jefferson-LVHN leadership argues that UnitedHealthcare has consistently engaged in bad-faith negotiation tactics, refusing to provide reimbursement rates that reflect the inflationary

costs of modern clinical delivery. Lehigh Valley Health Network alleges that UnitedHealthcare breached prior contractual covenants by unilaterally cutting clinical reimbursement rates by nearly 40% in 2021—an action currently being challenged through formal legal and judicial channels. Lehigh Valley Health Network asserts that UnitedHealthcare utilizes aggressive prior-authorization audits and excessive claim denials to delay payments and maximize corporate profitability at the expense of clinical care.

In response to the immediate panic surrounding the January Medicare Advantage termination, the Shapiro Administration intervened. Utilizing the state's regulatory authority, Pennsylvania Insurance Commissioner Michael Humphreys secured a federal Special Enrollment Period running from March 16 through April 30, 2026, specifically for UnitedHealthcare Medicare Advantage policyholders who utilized Lehigh Valley Health Network providers.

This regulatory intervention allowed displaced seniors to shop for alternative Medicare Advantage plans or transition back to traditional Original Medicare. Crucially, the Special Enrollment Period granted these seniors the right to purchase a Medicare Supplement policy without medical underwriting, protecting them from coverage denials based on pre-existing conditions.

The disruption also triggered federal political fallout, with U.S. Representative Rob Bresnahan, Jr. sending a formal letter to UnitedHealth Group and Jefferson Health leadership expressing deep frustration over the contract breakdown. Representative Bresnahan criticized both organizations for utilizing patients as negotiating leverage, highlighting the financial anxiety, disruption to care, and confusion experienced by constituents across Northeastern Pennsylvania. The congressional intervention demanded that both parties immediately resume negotiations and provide formal updates on steps taken to mitigate the clinical impact on families.

Market Realignment: St. Luke's and the Main Line Health Contrast

The UnitedHealthcare-LVHN fracture has triggered a major realignment of healthcare market share in Lehigh County, directly benefiting Lehigh Valley Health Network's primary regional competitor, St. Luke's University Health Network.

Anticipating the contract termination, UnitedHealthcare executed a multi-year in-network agreement with St. Luke's University Health Network. This agreement covers UnitedHealthcare's commercial, Medicare Advantage, Medicaid, and CHIP plans. To capture the displaced patient volume, St. Luke's established a dedicated clinical scheduling hotline (484-526-4440) specifically for UnitedHealthcare members seeking to transition their primary and specialized care away from Lehigh Valley Health Network.

While this competitive alliance provides a theoretical safety valve for network adequacy, the physical capacity of the regional healthcare system has been severely strained. Local clinical reports indicate that the influx of thousands of displaced Lehigh Valley Health Network patients has overwhelmed St. Luke's administrative and clinical infrastructure.

For example, pediatric cardiology and other high-demand specialties at St. Luke's are fully booked through late 2027, with many departments closed to new patients. This capacity constraint highlights a critical policy failure: contractual network adequacy on paper does not translate to actual clinical capacity in practice. Displaced patients requiring complex, continuous specialty care face long wait times, presenting a risk to clinical outcomes.

This prolonged disruption in Lehigh County stands in stark contrast to a concurrent contract dispute in southeastern Pennsylvania between UnitedHealthcare and Radnor-based Main Line Health. Main Line Health—which operates Lankenau Medical Center, Bryn Mawr Hospital, Paoli Hospital, Riddle Hospital, Bryn Mawr Rehab Hospital, and Main Line HealthCare—was scheduled to go out of network for UnitedHealthcare commercial, ACA marketplace, and Medicare Advantage members on July 1, 2026, potentially impacting 32,000 patients. However, on June 25, 2026, Main Line Health and UnitedHealthcare announced an agreement in principle. To finalize the multi-year contract terms, the parties extended active in-network access through July 14, 2026, ensuring no immediate disruption to patient care. According to Main Line Health's billing disclosures, the new agreement successfully adjusted UnitedHealthcare's reimbursement rates to reflect the rising cost of care while establishing concrete administrative protocols to reduce prior-authorization delays, claim denials, and clinical audit friction. This eleventh-hour resolution highlights that regional health networks utilize different leverage strategies, with Lehigh Valley Health Network executing a complete split while Main Line successfully negotiated a last-minute compromise.

Statutory Protections, Consumer Recourses, and PBM Oversight

Lehigh County consumers caught in the crossfire of the premium shock and the UnitedHealthcare-LVHN network split must navigate a complex landscape of state and federal regulatory protections. The primary legal shields include:

- **28 Pa. Code § 9.725 (Directory Accuracy Cap):** If an insurer's published provider directory contains an error—such as listing an out-of-network Lehigh Valley Health Network clinician as in-network—and a consumer relies on that directory to receive care, the consumer's financial liability is legally capped at standard in-network cost-sharing rates.
- **Act 252 of 2023 (Balance Billing Prohibition):** Under this state statute, any undisclosed out-of-network balance bill is legally presumed to be illegal. Providers are blocked from billing patients for the difference between the insurer's out-of-network reimbursement and the provider's gross charge unless explicit, advance consent was obtained.
- **Federal Continuity of Care Rules:** Under federal statutory guidelines, consumers who are actively undergoing a course of treatment for a serious and complex condition at the time of a network termination can petition UnitedHealthcare for transitional in-network coverage. This protection is not automatic and must be formally requested by the member and approved by UnitedHealthcare's clinical review board. If approved, UnitedHealthcare is required to cover the specific, ongoing treatment at in-network cost-sharing levels for up to 90 days, and longer in cases of active pregnancy through postpartum care. Qualifying clinical conditions include:
 1. Active pregnancy and postpartum care.
 2. Active cancer chemotherapy or radiation regimens.
 3. Scheduled non-elective major surgeries, including post-operative follow-up.
 4. Ongoing inpatient hospitalizations.
 5. Terminal illness requiring palliative care.

For services that do not meet these strict criteria, such as routine primary care, elective procedures, or annual diagnostic screenings, UnitedHealthcare is under no obligation to provide coverage. Members holding PPO plans can still access Lehigh Valley Health Network, but they

will be subject to substantially higher out-of-network deductibles, higher coinsurance rates, and elevated out-of-pocket maximums. For those holding restrictive HMO or EPO plans, out-of-network care at Lehigh Valley Health Network is completely uncovered, leaving the patient personally liable for 100\% of the clinical bill.

- **Federal Independent Dispute Resolution Misconceptions (45 C.F.R. § 149.510):** Policy researchers emphasize that many consumers are misled into believing they can file a federal Independent Dispute Resolution claim under the No Surprises Act to resolve billing disputes. This process is strictly a provider-versus-plan arbitration mechanism designed to resolve out-of-network payment rates between insurers and clinicians. Individual patients have no standing to initiate a dispute and must instead rely on state-level insurance department complaints (via 1-866-722-6675) or the free legal advocacy provided by the Pennsylvania Health Law Project (1-800-274-3258).
- **Pharmacy Benefit Manager Oversight (Act 77 of 2024):** Beyond medical network architecture, the Pennsylvania Insurance Department is enforcing new regulatory milestones under Act 77 of 2024 to increase transparency in the prescription drug supply chain. Pharmacy Benefit Managers are facing strict reporting deadlines in 2026: the first PBM Network Adequacy Report was due on April 1, 2026, and the first PBM Transparency Report is due on July 1, 2026. Regulators are legally mandated to post these transparency reports publicly within 60 days of receipt, exposing spread pricing, manufacturer rebate structures, and regional pharmacy access restrictions to public scrutiny.

number of Pennsylvanians qualifying for Medicaid by approximately 60,800. The executive budget notes that children in these impacted families would instead transition to the Children's Health Insurance Program, which is slated for a \$7.6 million funding increase to cover the expanding enrollment and adjust reimbursement rates for CHIP insurance carriers.

Furthermore, the budget includes \$1 million to implement the housing components of the state's newly approved Keystones of Health Medicaid demonstration waiver, alongside \$900,000 for the Food is Medicine nutritional initiative.

Outside of healthcare, the budget's progress remains blocked by a bitter dispute over public versus private education. House Democrats are prioritizing a \$565 million adequacy supplement distributed through the Ready to Learn Block Grant to target underfunded school districts.

In contrast, Senate Republicans are demanding an expansion of the Educational Improvement and Opportunity Scholarship Tax Credit programs, currently capped at \$575 million.

Republicans passed a bill expanding the scholarship credits by \$25 million, while House Democrats passed a competing bill that would implement strict accountability standards, requiring compliance audits by the state's auditor general and a 2% administrative fee on contributions to cover operational costs.

Other unresolved budget items include the Low-Income Home Energy Assistance Program, which provides cash heating grants ranging from \$200 to \$1,000 to vulnerable families, and a legislative push to expand Act 54 of 2024's municipal taxing authority. The proposed municipal tax bill would permit second-class townships to raise local tax limits via ordinance to fund essential staffing, training, and equipment costs for local firefighters and EMS companies.

Because the Pennsylvania Supreme Court ruled in 2009 that the federal Fair Labor Standards Act mandates the continued payment of state employee salaries during an impasse, the immediate operational pressure on lawmakers to pass a budget has been eliminated.

However, as the budget delay enters the second week of July, the suspension of discretionary state payments will begin to strain school districts, county-level mental health networks, early childhood intervention providers, and local non-profits, many of which must spend down cash reserves or take out high-interest bridge loans to survive.

For the 160,000 uninsured Pennsylvanians and the thousands of displaced patients in Lehigh County, this legislative gridlock represents a silent driver of clinical and financial vulnerability. Until a compromise is signed, the individual health insurance market in Pennsylvania will remain highly volatile, with regional networks operating under severe strain.

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